Understanding Substance Use, Misuse, Dependence & Addiction

Category 01: Understanding Alcohol & Drugs | Topic 04

A learning experience for youth, adults and families

Understand the difference between substance use, risky or harmful use, physical dependence, and substance use disorders. Explore fictional real-life situations, receive educational feedback, and learn when professional care matters.

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Publication status: Evidence-informed educational draft. Requires clinical review, youth-safeguarding approval, Rwanda-specific referral verification and multilingual validation before public use.

Format: This PDF contains the educational lesson and scripts for optional web interactions. The PDF itself is not an interactive application, diagnostic questionnaire or clinical treatment guide.

01. Start here: What will you learn?

A person may encounter alcohol or other substances through social life, prescribed treatment, curiosity, stress, or peer influence. These experiences should not be collapsed into one label. Understanding the terms helps us recognize health risks, communicate without stigma, and know when assessment is appropriate.

By the end of this topic, you can: distinguish key terms, recognize potential warning signs without diagnosing anyone, explain why tolerance and withdrawal do not automatically equal addiction, identify safe support options, and apply knowledge to fictional scenarios.

Core learning time: approximately 12–18 minutes, with optional activities. Learners can skip questions or go straight to help information. This time is an editorial estimate, not a measured result.

02. What does substance use mean?

Substance use means consuming alcohol or another substance. The term describes a behavior, not automatically a disorder. Substance use may carry health risks even if the person does not meet the criteria for addiction. For example, an individual may drink once and experience impaired judgment or an injury without having a substance use disorder. [1,2]

The context matters: a medication prescribed and used as directed is different from taking someone else's prescription for non-medical effects. A psychoactive substance can also be legal yet still pose

important health risks. The educational goal is not to assign moral labels, but to understand possible effects and harm.

Why we use the word "misuse" carefully

"Misuse" is sometimes used for taking medicines differently from medical instructions or for use associated with harm. It can be unclear or stigmatizing when applied loosely to people. In reader-facing content, prefer specific descriptions, such as "taking someone else's prescription," "using more than prescribed," or "continued alcohol use despite harm." This improves clarity and avoids implying that every substance-use behavior is clinically identical. [2,3]

03. Risky and harmful substance use

Some patterns raise the likelihood of future harm: using a substance before driving, combining alcohol with sedating medicines, or repeated use in settings where judgment and safety are essential. Other patterns have already caused harm, such as repeated injuries, illness, or major disruption to school, work, or relationships. The seriousness of harm is not measured by whether someone appears dependent. [1,2]

Example: A university student drinks heavily at a celebration and requires emergency assistance. The acute harm is important even if no dependence or ongoing disorder is identified. Another person may continue using a substance despite worsening health or family problems, which merits professional evaluation.

Key distinction

Risk does not equal diagnosis. A single incident can be dangerous; repeated problems can be a reason to seek care; neither should be used by a website to diagnose a person.

04. What are tolerance and physical dependence?

Tolerance describes a reduced effect from the same amount of a substance over time, or a need for more to obtain a similar effect. Tolerance can occur with some medicines even when taken appropriately and is not, by itself, proof of addiction. [3,4]

Physical dependence describes physiological adaptation to repeated exposure. When a substance is suddenly stopped or reduced, withdrawal symptoms may occur. These can differ widely according to the substance, dose, duration, health conditions, and other medicines. Dependence can develop during appropriate medical treatment and must not automatically be equated with compulsive use. [3,4]

Withdrawal is the set of physical or psychological symptoms that may occur following reduction or cessation of certain substances. Examples may include discomfort, sweating, sleep disturbance, or anxiety; some forms can be medically dangerous. Individuals should seek healthcare advice rather than abruptly changing prescribed medication or stopping heavy prolonged alcohol use without assessing withdrawal risks. [4,5]

05. What does addiction mean?

In everyday language, addiction usually refers to a pattern in which a person continues using a substance despite harmful consequences, with difficulty controlling use. Clinicians more often use diagnostic terms such as substance use disorder or alcohol use disorder. Disorders vary in severity and should be assessed by a qualified health professional. [1,2]

Possible features considered during assessment include using more than planned, unsuccessful attempts to cut down, craving, continuing despite problems, neglect of important roles, hazardous use, and sometimes tolerance or withdrawal. No single sign proves a diagnosis. Symptoms, duration, context and impairment matter. [2]

The distinction is important for a person taking long-term prescribed medication: withdrawal symptoms after stopping do not necessarily mean they are addicted. Equally, an individual may show serious compulsive substance use without prominent physical withdrawal. [3]

06. Compare the concepts

Concept	Simple meaning	What it does not prove
Use	A person consumes a substance	That the person has a disorder
Risky / harmful use	Use may increase risk or is causing harm	That dependence must be present
Tolerance	Same amount may produce less effect	That compulsive addiction exists
Physical dependence	Body adapts; withdrawal may follow reduction	That medication was misused
Substance use disorder	Clinically assessed problematic pattern	That recovery is impossible

07. Why problems can develop

Substance use disorders emerge from interacting biological, psychological, and social factors. Age of first exposure, individual vulnerabilities, stress, trauma, access to substances, peer norms, co-occurring mental health conditions, and the wider social environment may influence risk. These factors are not destiny, and none should be used to shame a young person or a family. [1,6]

Special attention to adolescence

Adolescence is a period of continuing brain and social development. Younger users should receive prevention-oriented, developmentally appropriate explanations and confidential, safeguarding-informed help when needed. The lesson must not invite children to share personal substance-use histories publicly or use a quiz as an informal screening process. [6]

What protects people?

Supportive relationships, safe school and community settings, credible information, coping skills, timely professional care, and prevention interventions based on evidence may lower risks. One-off warnings and fear-based campaigns alone should not be treated as sufficient prevention. [6]

08. Warning signs that merit attention

Potential warning signs include repeated difficulty cutting down, strong urges, missing important obligations, continuing use despite health or relationship problems, taking substances in unsafe circumstances, or increasing preoccupation with using them. Changes in sleep, mood or school performance may also be observed, but they have many possible explanations and must never be used alone to label a person. [2]

A useful approach is to observe specific changes, invite an optional private conversation, listen without accusation, and encourage professional assessment when concerns persist. Protect children from punishment, humiliation, forced public disclosure, and avoidable stigma.

09. Treatment and recovery: what help can look like

Substance use disorders are treatable health conditions. Care may include a structured assessment, brief intervention, counseling or other psychosocial treatment, medically supervised withdrawal when necessary, medicines for some disorders, management of co-occurring mental and physical health concerns, and continued community or family support. Treatment choice depends on the substance, clinical needs, available services, individual goals, and informed consent. [1,2]

Recovery is not identical for every person. Improvements in safety, health, family and work functioning, and sustained engagement with appropriate support may all matter. Recurrence of use can happen and should prompt reassessment of care rather than hopelessness or blame.

Rwanda context

Rwanda's Health Sector Strategic Plan V (2024/25–2028/29) prioritizes improving access to quality healthcare and addressing important noncommunicable disease risk factors. This provides policy context, not proof that this educational module is approved or licensed by the Government of Rwanda. Confirm current local substance-use care and referral routes before publication. [7]

10. Safe and respectful family support

A family member may say: "I've noticed you missed several commitments and seem unwell. I care about you. Would talking to a health professional help?" This is generally more useful than accusations or threats. Offer practical support with appointments where wanted, protect privacy, and avoid forcing promises that are difficult to keep. If violence, acute intoxication, or serious medical symptoms are present, prioritize immediate safety.

For minors, involve appropriately trained child-protection or healthcare personnel and follow applicable confidentiality and safeguarding rules. Do not suggest that a young person should solve a

dangerous situation alone.

11. Fictional situations: learning through real life

Scenario A — Pressure at school

A fictional 16-year-old student, Aline, is invited to drink with friends. They say everyone does it. She has not consumed anything and wants to refuse. Learning point: substance use is not necessary to belong. Aline can state her boundary and seek support from a trusted adult or safe friend if pressure continues.

Scenario B — Prescription medicine

Eric takes a medicine prescribed for a legitimate health condition. He notices discomfort when he forgets doses and worries this means addiction. Learning point: physical dependence can occur without addiction. Eric should discuss symptoms and any medication changes with his prescriber, not stop suddenly on his own.

Scenario C — Problems despite promises

Chantal, an adult, has repeatedly tried to reduce drinking but often drinks more than intended and has begun missing work. Learning point: patterns of impaired control and harm are reasons for a confidential assessment. A fictional story cannot establish Chantal's diagnosis.

Scenario D — Family concern

A parent notices their young adult child is increasingly withdrawn and missing responsibilities. Learning point: these observations are nonspecific. A respectful conversation and professional support are better than assuming a substance use disorder or conducting a public confrontation.

Scenario E — Considering stopping alcohol

A person has consumed large amounts of alcohol over a long period and wants to stop immediately. Learning point: the next step is prompt assessment of possible withdrawal risks. Severe alcohol withdrawal can be dangerous and may require supervised medical care. [5]

12. Optional interactive learning - question and feedback scripts

These examples will become interactive choices on the website. The PDF presents the question, options, recommended response, and explanation. They are not a diagnostic tool and collect no personal history. A user may skip or retry.

Activity 1 - Identify the term

Question: A patient has withdrawal symptoms after suddenly stopping a correctly prescribed medicine. What can we conclude?

A. This always proves addiction.

B. Physical dependence is possible, but addiction cannot be assumed.

C. A person should always stop treatment immediately.

Recommended answer: B. Physical adaptation and addiction are distinct. Medication decisions require clinical guidance. [3,4]

Activity 2 - Recognize risk

Question: Someone drives after drinking but has no history of dependence. Which statement is accurate?

A. There is no health risk without addiction.

B. Impairment and injury can occur without addiction.

C. A quiz can verify driving ability.

Recommended answer: B. A single episode of use can create danger. Do not drive or ride while impaired.

Activity 3 - Respond without stigma

Question: A friend says they cannot reduce substance use despite repeated harm. What is a helpful response?

A. Shame them publicly.

B. Encourage confidential professional assessment and listen.

C. Diagnose them from the story.

Recommended answer: B. Support focuses on respect, specific concerns, and appropriate care.

Activity 4 - Youth pressure

Question: A student refuses alcohol but friends continue insisting. What might help?

A. Accept to prove maturity.

B. State a boundary and seek a safe supporter.

C. Take unknown medication instead.

Recommended answer: B. Personal boundaries and supportive adults can reduce pressure; the young person should not be required to disclose details publicly.

Activity 5 - Withdrawal safety

Question: A person with a history of prolonged heavy drinking wants to quit immediately and is worried about shaking. What is the safest advice?

A. Attempt unsupervised detoxification.

B. Seek prompt medical advice about withdrawal.

C. Replace alcohol with sedative pills from a friend.

Recommended answer: B. Withdrawal can require medical assessment and treatment; do not give self-directed detox regimens. [5]

13. When urgent help is necessary

Seek emergency medical assistance when someone cannot be awakened, has slow or irregular breathing, seizures, major confusion, or other signs of serious poisoning. Contact an appropriate local emergency provider or go to the nearest emergency department. Do not leave an unresponsive person unattended while arranging help.

For a person who may be physically dependent on alcohol, suddenly stopping after prolonged heavy use may provoke serious withdrawal symptoms, including seizures or delirium. Seek qualified healthcare advice rather than attempting unsupervised withdrawal. [5]

Service accuracy: Before public release, VUGA NDAKUMVA must verify emergency numbers, facility details, hours, age-specific pathways and referral procedures with appropriate Rwanda authorities. Do not invent local contacts.

14. Key takeaways

• Substance use is a behavior; it is not automatically a disorder.

• A single episode may cause harm even without dependence or addiction.

• Tolerance and physical dependence can occur without addiction.

• Substance use disorders involve a broader pattern assessed by healthcare professionals.

• Young people need prevention, trusted support, and protection from stigma.

• Treatment and recovery are possible; urgent withdrawal and poisoning risks need medical attention.

15. Publishing the interactive lesson accessibly

Element	User experience requirement
Reading	Short sections, plain language, visible source links; no forced age gate
Scenario	Fictional stories; no disclosure of personal substance use
Choices	Large tap targets and keyboard support; answer feedback explained
Video / illustration	Optional, captioned, transcript and meaningful alternative text
Low-data	Full lesson accessible without downloading video
Safety	Get Help always available without completing lessons or quizzes
Privacy	Anonymous learning by default; no unnecessary sensitive data

16. Clinical and safeguarding review before publication

• Verify terminology against current WHO/UNODC and relevant Rwanda clinical guidance.

• Have an addiction medicine or mental health professional check risk and withdrawal statements.

• Have a youth safeguarding specialist review all adolescent scenarios and choices.

• Confirm current Rwanda care pathways and legally applicable child-privacy requirements.

• Validate Kinyarwanda and French translations with trained human reviewers.

• Test comprehension and screen-reader/keyboard accessibility with representative users.

• Document content owner, source dates, approval, next review date, and revision history.

17. Sources and further reading

[1] WHO / UNODC (2020). International Standards for the Treatment of Drug Use Disorders.
https://www.who.int/publications/i/item/international-standards-for-the-treatment-of-drug-use-disorders

[2] US National Institutes of Health. Substance Use Disorders: Health Topic.
https://health.nih.gov/health-topics-a-z/substance-use-disorder

[3] National Institute on Drug Abuse. Misuse of Prescription Drugs (dependence, addiction, tolerance).
https://nida.nih.gov/sites/default/files/2609-misuse-of-prescription-drugs_0.pdf

[4] MedlinePlus. Substance Use Disorder.
https://medlineplus.gov/ency/article/001522.htm

[5] WHO. Management of Alcohol Withdrawal (mhGAP evidence centre).
https://www.who.int/teams/mental-health-and-substance-use/treatment-care/mental-health-gap-action-programme/evidence-centre/alcohol-use-disorders/management-of-alcohol-withdrawal

[6] WHO / UNODC (2018). International Standards on Drug Use Prevention, second edition.
https://www.who.int/publications/i/item/international-standards-for-drug-use-prevention-second-edition-2018

[7] Rwanda Ministry of Health. Health Sector Strategic Plan V, 2024/25–2028/29.
https://www.moh.gov.rw/strategic-plans-priorities

Evidence status: References support a general health education draft. This module is not a diagnosis, prescription, detoxification protocol, or confirmation of government endorsement.